

Addis Ababa University
College of Education and Behavioral Studies
Department of Special Needs Education

Inclusiveness

Chapter 3

Identification of the Impact of Disability and Differentiated Services

- 1. What are the impacts of disability?**
- 2. What are services given for PWD?**

3.1. The Impact of Disability and Vulnerability on Daily Life

- People respond to disabilities in different ways. Some **react negatively**.
- Thus, in this situation the quality of **life** of PWDs will be **negatively affected**.
- Others prefer to **focus on their abilities** as opposed to their disabilities and continue to **live a productive life**.
- There are several factors that affect and impact of the life an individual with disability.

The following are considered as the most known factors in determining the impact of disability on an individual.

3.1.1.The Nature of impairment

- Impairment can be acquired as a result of an accident, or disease that may be **congenital**/ or **acquired** (**pre-natal**, or **post-natal**).
- If the impairment is acquired, it is more likely to cause a negative reaction than a congenital impairment.
- The **acquired** impairment requires more rehabilitation service than the congenital ones.

3.1.2. The Individual's Personality

- The individual personality can be typically **positive** or **negative**, **dependent** or **independent**, **goal-oriented** or **laissez-faire**.
- Someone with a positive outlook is more likely to embrace/ **hold**/ impairment than someone with a negative outlook.
- Someone who is independent will continue to be independent and someone who is goal-oriented will continue to set and pursue goals.

3.1.3. The Meaning of the impairment to the Individual

- Does the individual define himself/herself by his/her looks or physical characteristics?
- If so, he/she is more likely to feel defined by his/her impairment and thus it will have a negative impact.

3.1.4. The Individual's Current Life Circumstances/ Situations/

- The influence of this situation manifests through the following factors:
 - The condition that the individual is **independent** or **dependent** on others (parents) in his/her living;
 - The **economic** and **academic** status of the individual with disability or his/her **caregivers**;
 - If the individuals with impairments are happy with their current life circumstance, they are more likely to embrace their impairment, whereas, **if they are not happy** with their circumstances, **they often blame their impairment**.

3.1.5. The Individual's Support System

- The individual's support from **family**, a significant other, **friends**, or **social** groups contribute to **minimize** the impact of impairment on their overall personality

Common effects of impairment/disability may include:

- Limited to health conditions of the person;
- Mental health issues including anxiety and depression;
- Loss of freedom and independence;

- Frustration and anger at having to rely on other people;
- practical problems including transport;
- Limited choice and activities;
- Difficult to accessing buildings;
- High unemployment level;
- Problems with learning and academic study;
- Loss of self-esteem and confidence especially in social situations.

- But most of these **negative effects** do exist due to restricted environments, **not as a result of direct impact of the impairment.**
- Persons with disabilities are **diverse** and **heterogeneous**, while stereotypical views of disability emphasize wheelchair users and a few other “classic” groups such as blind people and deaf people.
- Disability encompasses the child **born with a congenital** condition such as cerebral palsy or the young soldier who loses his leg , or the middle-aged woman with severe arthritis, among many others.
- Health conditions can be **visible** or invisible; **temporary** or long term.

- Note that many people with disabilities do not consider themselves to be **unhealthy**.
- Persons with disabilities have **diverse personal factors** with differences in **gender**, **age**, **language**, **socioeconomic** status, **ethnicity**, or **cultural heritage**.
- Women with disabilities experience the **combined** disadvantages associated with gender as well as disability, and may be **less likely to marry** than non-disabled women.
- People who experience **mental health** conditions or **intellectual impairments** and severe/multiple impairment appear to be more **disadvantaged** in many settings than those who experience **physical or sensory impairments**.

3.1.6. Economic Factors and Disability

- There is clear evidence that people living with poverty are exposed to health problems that may cause impairment and disabling environment.
- This is true even in advanced economies.
- The lack of resources can adversely affect the ability of an individual to function with a disabling condition.
- For example, someone with an amputated limb who has little money or poor health insurance may not be able to obtain a proper artificial limb, in which case the absence of the limb may then force the individual to withdraw from jobs that require these capacities.

- Similarly, **economic resources** can limit the options and abilities of someone who requires personal assistance services or certain physical accommodations.
- The individual also may not be able to access the appropriate **rehabilitation services to reduce the degree of potential disability** either because they cannot afford the services themselves or cannot afford the cost of **specialized transportation services**.
- The advanced economy of the communities and the country at large significantly make available **enabling environment** in **infrastructure, public services** and **job opportunities** for the participation of PWDs.
- In the countries having advanced economy, governments may **apply disability compensation policy**.

3.1.7. Political Factors and Disability

- The **political system**, through its role in **designing public policy**, can and does have a profound impact on the extent to which impairments and other potentially disabling conditions will result in disability.
- If the **political system** is well enforced it **will profoundly improve the prospects of people with disabling conditions** for achieving a much **fuller participation** in society, in effect **reducing the font of disability** in work and every other domain of human activity.
- Thus, the political good will of the government that manifests at **policy and implementation** level creates **enabling environment** for full and effective participation of PWDs in all aspects of life of the society.

3.1.8. Psychological Factors of Disability

- There are five constructs that may result in psychological influence on the psychological makeup of PWDs in reference to **disabling conditions**.

Do you think that disabling conditions may have psychological influence?

I. Social Cognitive Processes

- Cognition consists of thoughts, feelings, beliefs, and ways of viewing the world, others, and ourselves.
- **Three interrelated cognitive processes** have been selected to illustrate the direct and interactive effects of cognition on disability.
- These are **self-efficacy beliefs**, **psychological control**, and **coping patterns** which all these are socially constructed.

II. Self-Efficacy Beliefs

- Self-efficacy beliefs are concerned with whether or not a person believes that he or she can accomplish a desired outcome (Bandura, 1977, 1986).

III. Psychological Control

- Psychological control, or control beliefs, is akin / **similar** /to self-efficacy beliefs in that they beliefs regarding one's ability to **exert control** or **change a situation**.

IV. Coping patterns

- **Coping** patterns refer to **behavioral** and **cognitive** efforts to **manage** specific **internal** or **external** demands that tax or exceed a person's resources to adjust.

V. Personality Disposition/ Temperament/

- Optimism in contrast to pessimism is the general tendency to view the world, others, and oneself favorably.
- People with an optimistic orientation rather than a pessimistic orientation are far better across several dimensions.
- Optimists tend to have better self-esteem and less hostility towards others and tend to use more adaptive coping strategies than pessimists.

3.1.9. The Family and Disability

- The family can be either an **enabling** or a **disabling** factor for a person with a disabling condition.

How and when families considered as disabling factor ?

- **Some families promote dependency.**
- Although most people have a **wide network of friends**, the networks of people with disabilities are more likely to be dominated by **family members**.
- Even among people with disabilities who maintain a large network of friends, family relationships often are most central and **families often provide the main sources of support.**
- This support may be **informational** (providing advice or referrals), or **emotional** (giving love and support).

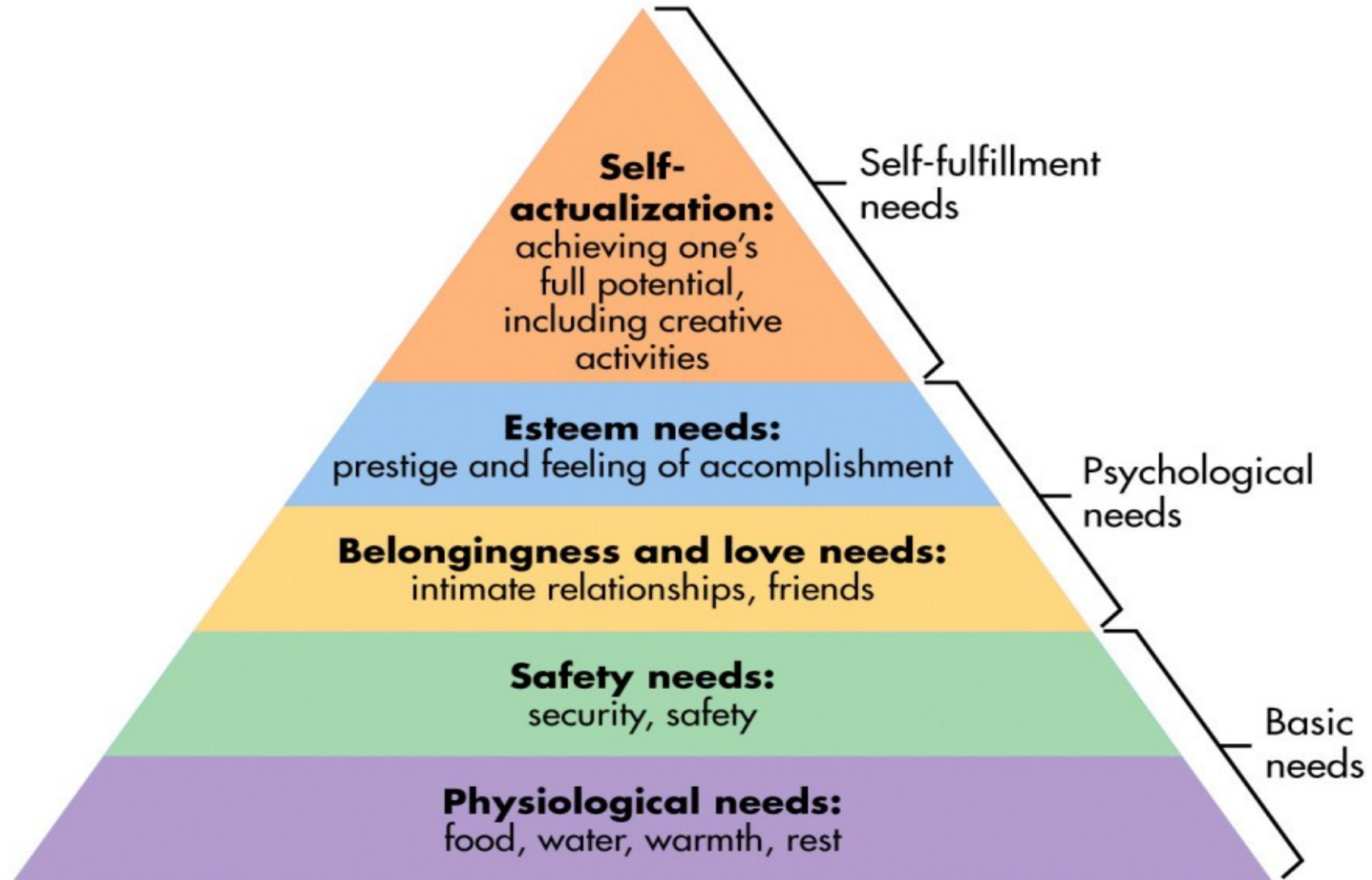
- Families can also fulfill their responsibilities in meeting the special needs of their children with disabilities by making available specialized services and assistive devices and technologies
- It is important to note, however, that families may also be disabling.
- Others fatalistically accept functional limitations and conditions that are amenable to change with a supportive environment.

3.2. Special Needs of Persons with Disabilities and other Vulnerable Groups

- The special needs of PWDs are **emanated** from different factors.
- These are:
 - Personal experience of individuals with impairments;
 - Type of the impairment;
 - Severity /degree of the impairment; and
 - Onset of the impairment.
- Hence, professionals as per their discipline should give response differently to the needs of PWDs considering these realities.

- Analyzing the human beings, Maslow has identified five categories of needs, with different priority levels, in the following order:
 1. Survival (physiological);
 2. Safety;
 3. Social needs;
 4. Esteem; and
 5. Self-actualization (fulfillment).
- Maslow's model is also valid for persons with disabilities and other vulnerable groups, whose needs are regular and or similar to those of ordinary persons.
- It is obvious that survival is a priority need for PWDs due to the existing disabling condition such as lack of opportunity for personal hygiene, feeding and Adaptive Daily Living skills (ADL).

Abraham's Maslow Hierarchy



- Persons with disabilities and other vulnerable groups have socio-emotional, psychological, physical and social environmental and economic needs in general.
- **However**, the following are the basic and special needs of PWDs and other vulnerable groups needed to be available to **ensure their equal participation in all activities of the society**:
 - Full access to the environment (towns, countryside & buildings);
 - Accessible transport system;
 - Technical aids and equipment;

- Accessible/adapted housing;
- Personal assistance and support;
- Inclusive education and training;
- Adequate income;
- Equal opportunity for employment;
- Appropriate and accessible information;
- Advocacy (towards their human right);
- Counseling;
- Appropriate and accessible health care;
- Assistive appliances.

3.2.1. Belongingness and disability

- Family relationships as a means of **connecting to community** and being known by others, and knowing others outside the family are important.
- Different kinds of relationship contributed to this sense of belonging, ranging from the more superficial nodding acquaintances to specific informal support from known others, to the intimacy of close friends and kin.

- Historically for people with disabilities, **rurality was once** the site of exclusion, rather than belonging, where identity and gender were disregarded in favor of ensuring protection of people with disabilities and of the society in which they lived.
- People with disabilities and marginalized groups feel **isolated**.
- Some persons with disabilities have actively sought / wanted/ to **migrate** to urban environments, to escape from the confines and constraints of small rural environments and to build broader social networks away from the farm.

3.3. Disability Inclusive Intervention and Rehabilitation Services

- A '*One-size-fits-all*' approach to provide services for persons with disabilities and vulnerable groups is no longer enough.
- Including people with disabilities in everyday activities and encouraging them to have roles similar to peoples who do not have a disability is *disability inclusion*.
- This involves more than simply encouraging people; it requires making sure that adequate policies and practices are in effect in a community or organization.

- Inclusion should lead to **increased participation** of PWDs in socially expected life roles and activities such as: being a student, worker, friend, community member, patient, spouse, partner, or parent.
- Disability inclusion means **provision of differentiated services** for persons with disabilities and other vulnerable groups.
- Differentiated service means a multiple service delivery model that can **satisfy** the most needs of persons with disabilities and other vulnerable groups.

- This includes the availability of accommodative public services in **infrastructure, health care, education, social protection** etc.
- Persons with disabilities and other vulnerable groups are often **excluded** (either directly or indirectly) from development processes and humanitarian action because of physical, attitudinal and institutional barriers.
- The effects of this exclusion are increased inequality, discrimination and marginalization.

3.3.1. Definition and components of Rehabilitation Intervention

- Rehabilitation is a process designed to **optimize** function and **improve the quality of life** of those with disabilities.
- There are general underlying concepts and theories of rehabilitation interventions.
- Examples of these theories and concepts include movement and motor control, human occupation models, education and learning, health promotion and prevention of additional and secondary health conditions, neural control and central nervous system plasticity, pain modulation, development and maturation, coping and adjustment, biomechanics, linguistics and pragmatics, resiliency and self-reliance, auditory processing, and behavior modification.

- The conduct of rehabilitation intervention is not a simple process. It involves **multiple participants**, and it can take on many forms.
- The following is a description of the individual components that, when combined, comprise the process and activity of rehabilitation:
 - Multiple Disciplines;
 - Physicians;
 - Occupational Therapists;
 - Physical Therapists;
 - Speech and Language Therapist;
 - Audiologists;
 - Rehabilitation Nurses;

- Social Workers;
- Case Managers;
- Rehabilitation Psychologists;
- Neuropsychologists;
- Therapeutic Recreation Specialists;
- Rehabilitation Counselors;
- Orthotists and Prosthetists;
- Additional Rehabilitation Professionals;
- Persons with the Disability and His or Her Family;
- Community Based Rehabilitation Workers.

3.3.2. Strategies to Disability Inclusive Intervention and Rehabilitation

A) Prevention

- This includes **primary** Prevention (**Action intended to avoid the causes of impairment**), **Secondary** prevention (**Early Intervention**) and **tertiary** prevention (Rehabilitation).

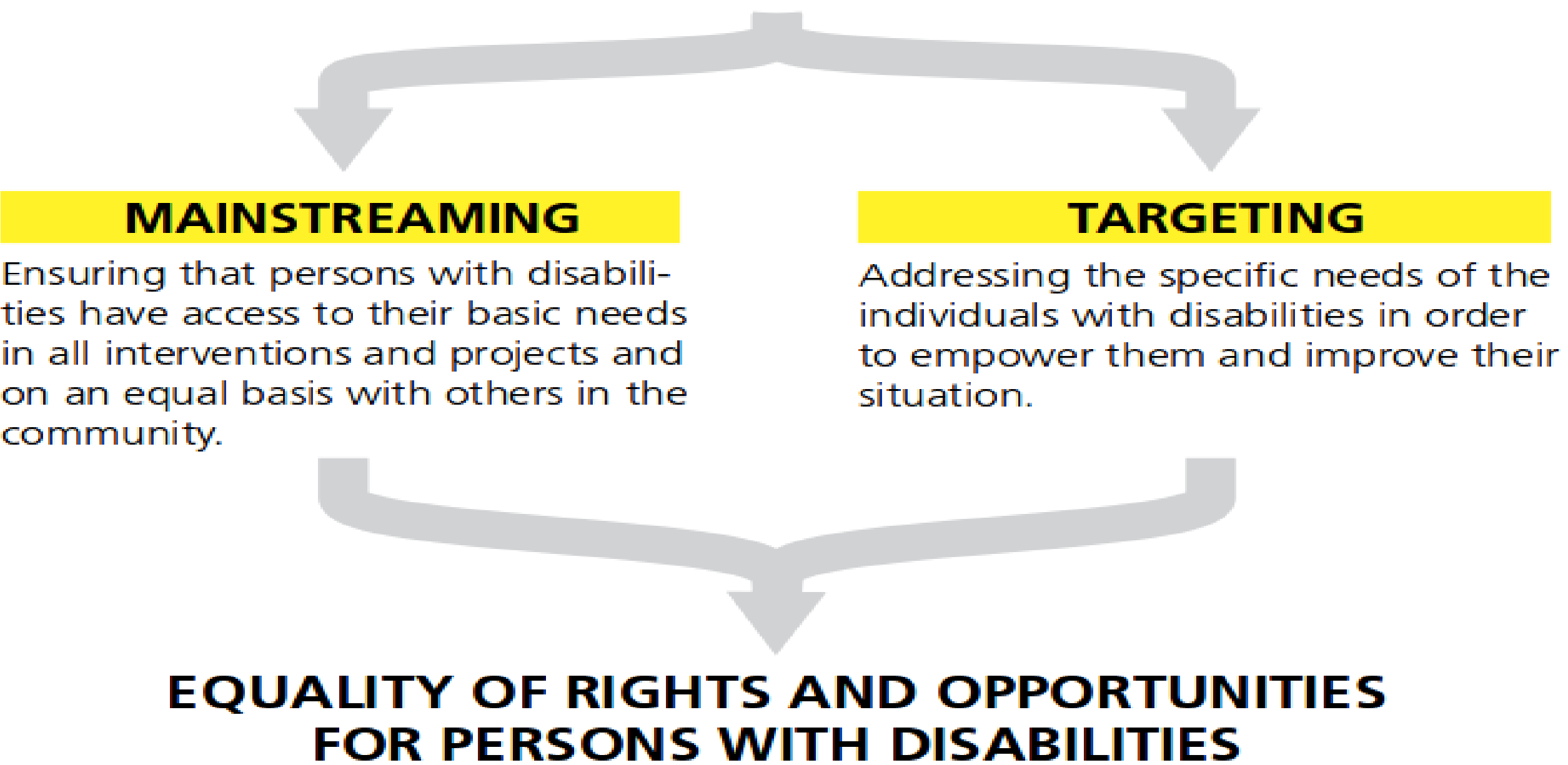
B) Implementing the Twin-track Approach

One of the strategies relevant for the implementation of inclusion in terms of disability is Twin-track approach.

The twin-track approach involves:

1. Ensuring all mainstream **programs and services** are **inclusive** and **accessible** to persons with disabilities and other vulnerable groups time; and
2. Providing **targeted disability-specific** support to persons with disabilities.

TWIN-TRACK APPROACH



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graph TD; A[TWIN-TRACK APPROACH] --> B[MAINSTREAMING]; A --> C[TARGETING]; B --> D[EQUALITY OF RIGHTS AND OPPORTUNITIES FOR PERSONS WITH DISABILITIES]; C --> D;
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The diagram illustrates the Twin-Track Approach. At the top, the title 'TWIN-TRACK APPROACH' is centered. A large grey arrow points downwards from this title, splitting into two parallel paths. The left path leads to a yellow box labeled 'MAINSTREAMING', which contains the text: 'Ensuring that persons with disabilities have access to their basic needs in all interventions and projects and on an equal basis with others in the community.' The right path leads to a yellow box labeled 'TARGETING', which contains the text: 'Addressing the specific needs of the individuals with disabilities in order to empower them and improve their situation.' Both of these boxes have grey arrows pointing downwards from their bottom centers, which then merge into a single large grey arrow pointing to the final outcome at the bottom: 'EQUALITY OF RIGHTS AND OPPORTUNITIES FOR PERSONS WITH DISABILITIES'.

MAINSTREAMING

Ensuring that persons with disabilities have access to their basic needs in all interventions and projects and on an equal basis with others in the community.

TARGETING

Addressing the specific needs of the individuals with disabilities in order to empower them and improve their situation.

**EQUALITY OF RIGHTS AND OPPORTUNITIES
FOR PERSONS WITH DISABILITIES**

C) Implement Disability Inclusive Project/ Program

- As **public service** provider all sectorial **strategies, program, projects** and **services** must be disability-inclusive.
- The sectors operations should be largely framed within broad programs, making it very important to ensure that disability inclusion is reflected in program **strategies and design documents**.
- This in turn ensures the mainstreaming and implementation of disability issues at **program level**.
- However, persons with disabilities are often not considered in crucial stages of most sectorial and developmental program and projects because of lack of awareness about the special needs of people with disabilities, and other vulnerable groups.

- The following **tips** will help to **overcome the challenges** as key considerations for including persons with disabilities in all program.

project cycle management stages:

Assessment, **Planning**, **Implementation** and **Monitoring**, and **Reporting/Evaluation**:

- Education and vocational training;
- Health;
- Relief and social services;
- Infrastructure and camp improvement, shelter, water and sanitation and environmental health;
- Livelihoods, employment and microfinance;
- Protection;
- Humanitarian and emergency response.

D)Community Based Rehabilitation (CBR)

- Community Based Rehabilitation (CBR) is a strategy crafted for developing countries with an intention to bringing children with disabilities who are kept at home to public services such as: education, self-employment, health care etc. through providing them multi-dimensional *home-based rehabilitation services*.
- For this purpose, there is a need to recruit and deploy CBR workers from and within the local communities respectively.

E) Inclusiveness and Assistive Technology

- There are various organizational definitions for **assistive technology**:

- ❖ The international standard ISO 9999 defines AT (referring to **AT** as “**technical aid**”) as “any product, instrument, equipment or technical system used by a disabled person, especially produced or generally available, **preventing**, **compensating**, **monitoring**, **relieving** or **neutralizing** the impairment, disability or handicap”.

- The use of **devices**, **computers**, **robots**, and other established assistive technology (AT) can potentially increase the autonomy of people with disabilities and other vulnerable groups, by compensating for physical limitations and circumventing difficulties with normal activities of daily living (ADL).
- In modern societies, persons with disabilities, and other marginalized and other vulnerable groups can attain some components of wellbeing such as access to services using assistive technology (AT).
- Other components, such as **freedom** of **navigation** and **travel**, are much more difficult because of environmental obstacles encountered by the disabled.

- Surgery, generic therapy, rehabilitation, human assistance, and the use of assistive technology (AT) help disabled people cope with their disabilities.
- Unfortunately, such assistance is not always available and not necessarily cost-effective.
- AT can increase the autonomy, independence, and quality of life for Persons with disabilities and other vulnerable groups and can also enable the integration of social, professional, and environmental aspects of life for Persons with disabilities and other vulnerable groups.

- Assistive technology affords Persons with disabilities and other vulnerable groups' greater equality of **opportunity**, by enhancing and expanding their **communication**, **learning**, **participation**, and achievement with higher levels of independence, wellbeing, and quality of life.
- Assistive Technology varies from low-tech devices such as a **cane** or adapted **loop**, to high-tech systems such as **assistive robotics** or **smart spaces**.
- Assistive technology encompasses all systems that are designed for Persons with disabilities and other vulnerable groups, and that attempt to compensate the handicapped.

3.4. Implement Inclusive Job Opportunities and Employment

- The right to work is fundamental to being a full and equal member of society, and it applies to all persons, regardless of whether or not they have a disability.
- A decent job in the open labor market is a key bulwark against poverty.
- It also enables people to build **self-esteem**, form social relationships, and to **gain** skills and knowledge.
- Despite the fact that the majority of jobs can be performed by individuals with disabilities, the pathways to their employment are often strewn with barriers.

- Studies indicated that there is **high rate** of unemployment within working age PWDs compared with non-disabled productive forces even in industrialized countries.
- Most of PWDs and other vulnerable groups are forced to work in **informal works** without legal contract and job security.
- Even when persons with disabilities are formally employed, they are more likely to be in **low-paid**, low-level **positions** with poor prospects for **career** development.
- The recently adopted 2030 Agenda for Sustainable Development calls on governments around the world to promote **full employment** and **decent work** for all, including persons with disabilities and other vulnerable groups.

- **Persons with disabilities may be prevented from working due to the following barriers:**
 - Inaccessible transportation services;
 - Lack of accessible information and communications services;
 - Preference of employers for candidates without disabilities;
 - Legal stipulations that prevent individuals with particular impairments from working in certain fields;
 - Discouragement of family and community members;
 - Lack of opportunity to education and vocational training to meet the requirements of labor market;
 - Benefit trap.

- The following are **key strategies** needed for the promotion of job opportunity for PWDs and other vulnerable groups.
 - Anti-Discrimination Legislation;
 - Vocational Education and Training;
 - Wage Subsidies;
 - Workplace Accommodation Schemes;
 - Workers' Compensation;
 - Quota Systems;
 - Sheltered Workshops;
 - Private Sector Initiatives;
 - Employer Networks;

- Support Disability-Inclusive Business;
 - Support Persons with Disabilities in the Workplace;
 - Building a More Inclusive Society;
 - Improve Data Collection on Disability and Employment.
- Regardless of those challenges that may affect the stamina and motivation of PWDs, studies confirmed that PWDs seriously want to work to:
 - Livelihood;
 - Ensure their social contacts; and
 - Develop their self-esteem.

Activities

1. Do you believe that PWDs have the same need? Why?
2. Why intervention is needed for PWDs and other vulnerable groups?
3. What is rehabilitation? And why and for whom it is needed?
4. In your point of view why TA is needed for person with disabilities?
5. How would you evaluate the involvement of the community (CBR) in your areas that you were born and grew up?